

Adverse Drug Reaction Reporting

Policy Number	POL-PH-036	Owner / Department	Pharmacy / Patient Safety
Effective Date	2023-10-01	Version	2.1
Last Revised	2026-03-20	Next Review	2028-03-20
Approved By	Pharmacy & Therapeutics Committee	Applies To	All prescribers, pharmacists, and nurses who observe, treat, or document suspected adverse drug reactions

1. Purpose

To standardize recognition, immediate treatment, documentation, and reporting of adverse drug reactions (ADRs), including internal surveillance and external reporting to the FDA MedWatch program, per CMS pharmaceutical-services requirements.

2. Definitions

- **Adverse drug reaction (ADR)** — a noxious and unintended response to a medication used at normal doses (distinct from a preventable medication error).
- **Serious ADR** — an outcome that is fatal, life-threatening, results in or prolongs hospitalization, causes disability, or requires intervention to prevent permanent harm.
- **Anaphylaxis** — a severe, rapid-onset, potentially fatal systemic hypersensitivity reaction.
- **MedWatch** — the FDA program for reporting adverse events and product problems.

3. Policy and Procedure

All suspected ADRs are treated clinically, documented in the record (including updating the allergy/intolerance list), and reported to Pharmacy for assessment and pharmacovigilance. Serious and unexpected reactions are reported externally to FDA MedWatch (and to the manufacturer where applicable).

1. Stop the suspected medication and provide clinically indicated treatment.
2. Notify the prescriber and obtain orders for ongoing management.
3. Update the allergy/intolerance list with the reaction type and severity.
4. File an ADR report to Pharmacy; assess causality and preventability.
5. Submit serious or unexpected reactions to FDA MedWatch per the facility process.

IMPORTANT — For suspected anaphylaxis, stop the drug and give intramuscular epinephrine without delay, call for emergency assistance, and support the airway and circulation. Do not wait for a provider order to administer epinephrine under the approved anaphylaxis protocol.

4. ADR Severity and External Reporting

ADR type	Example	External reporting
Life-threatening	Anaphylaxis; Stevens-Johnson syndrome.	FDA MedWatch; manufacturer.
Serious	Reaction prolonging stay or needing intervention.	FDA MedWatch.
Non-serious	Mild self-limited rash; transient nausea.	Internal pharmacovigilance log.

5. Severity & Reporting

Classify ADRs on the POL-RM-013 scale. Because an ADR at normal doses is generally not a preventable error, the NCC MERP index in POL-PS-003 applies only when a medication error contributed to the reaction.

Severity	Example	Notify / timeframe
SEV-1	Fatal or life-threatening reaction (anaphylaxis, SJS/TEN).	Provider, Nursing Supervisor & Risk Mgmt within 1 hour.
SEV-2	Reaction requiring intervention or prolonging hospitalization.	Provider & Risk Mgmt within 4 hours.
SEV-3 / Near Miss	Mild, self-limited reaction; reaction averted by allergy-alert interception.	Event report within 24 hours.

6. Documentation Requirements

- Suspected medication, dose, and route, and the reaction onset and description.
- Treatment given and the patient's response.
- Updated allergy/intolerance list entry with severity.
- Causality/preventability assessment and any MedWatch submission reference.

7. References

- FDA MedWatch — The FDA Safety Information and Adverse Event Reporting Program.
- CMS Conditions of Participation, 42 CFR 482.25 (pharmaceutical services, ADR reporting).
- ISMP and pharmacovigilance best-practice guidance.
- Related: POL-RM-013 (severity & reporting); POL-PS-003 (medication-error reporting).